3.10 Hemothorax

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel dizzy and short of breath." **Objective** • Blood coming out of the chest tube • BP 92/58 mmHg, HR 118/min • Skin pale and cold	**ACTUAL** **Deficient fluid volume** related to blood loss into the pleural space as evidenced by hypotension, tachycardia and bloody chest drainage.	Within 12 hours, vital signs stable and drainage falling.	• Vital signs every 15–30 minutes until stable. • Record chest drain output every hour. • Report drainage above the surgeon's limit (often more than 200 mL/hour). • Give IV fluids and blood as prescribed. • Follow the Hb.	Vital signs stable, drainage less: **goal met**.
Subjective • "I cannot breathe deeply." **Objective** • Breath sounds reduced on the affected side • SpO_2 90%	**ACTUAL** **Impaired gas exchange** related to lung compression by blood as evidenced by hypoxemia and reduced breath sounds.	SpO_2 kept at 94% or more.	• Give oxygen as prescribed. • Sit him semi-Fowler. • Keep the chest drain patent.	SpO_2 on target: **goal met**.
Subjective • *No specific complaint* **Objective** • Still bleeding	**RISK** **Risk for shock** as evidenced by risk factors: hypovolemia from continuing blood loss.	Perfusion adequate: MAP above 65 mmHg, urine output at least 0.5 mL/kg/hour.	• Put in two large-bore IV cannulas as ordered. • Record urine output every hour. • If bleeding goes on, get him ready for thoracotomy.	Perfusion adequate: **goal met**.